GOMTI RIVERSIDE HOSPITAL & CANCER CENTRE
Surgical Oncology | Gastro-surgery
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Reg. No. UP/LKO/HOSP/2010/0462

OPERATION NOTES

Patient: Mr. Anil Kumar Mishra, 55 Y / M, IP No. GRH/26/07712

Date of surgery: 02/06/2026

Surgeon: Dr. Siddharth Awasthi Assistant: Dr. Nitin Saxena Anaesthetist: Dr. Rakhi Tandon

Anaesthesia: General anaesthesia

Procedure: Laparoscopic low anterior resection with diverting loop ileostomy

Indication: Carcinoma mid-rectum, after neoadjuvant treatment

Findings

Residual thickening of the mid-rectum with fibrosis in the mesorectum. No peritoneal or hepatic deposits. Splenic flexure mobilised; inferior mesenteric artery ligated at its origin.

Steps

Four-port technique. Total mesorectal excision down to the pelvic floor. Rectum divided with a linear stapler; colorectal anastomosis with a circular stapler, air-leak test negative. Loop ileostomy fashioned in the right iliac fossa. Pelvic drain placed. Blood loss about 150 mL.

Specimen sent for HPE.

Post-operative: nil by mouth for 24 hours, then sips; stoma care teaching to be started on day 2.

Dr. Siddharth Awasthi
M.S., M.Ch.
Reg. No. UPMC-2009/03/5521
Surgeon

SYNTHETIC DOCUMENT — FOR DEMONSTRATION ONLY